

Women's Specialists of Clear Lake

Dr. Geoffrey H. Klein, MD · FACOG · DABOM · 400 W Medical Center Blvd, Suite 300, Webster, TX 77598 · (281) 557-0300

VBAC: Vaginal Birth After Cesarean

If you have had a prior cesarean, you may have a choice -- a planned repeat cesarean or a trial of labor (TOLAC) toward a VBAC

WHAT IS VBAC AND TOLAC?

- **VBAC** = a vaginal birth after a previous cesarean delivery
- **TOLAC** = the **trial of labor** attempted in order to achieve a VBAC
- If TOLAC is successful, you have a vaginal birth (a VBAC)
- If TOLAC is not successful, you will have a **repeat cesarean** delivery
- Most appropriate candidates with one prior **low transverse** cesarean have about a **60-80% chance** of a successful VBAC
- The decision is shared -- based on your history, this pregnancy, and your goals

BENEFITS OF A SUCCESSFUL VBAC

- ✓ **No abdominal surgery** and no surgical scar from this birth
- ✓ **Shorter recovery** -- usually back to normal activities sooner
- ✓ **Less blood loss** than a cesarean
- ✓ **Lower risk of infection**
- ✓ **Earlier bonding and breastfeeding** start in many cases
- ✓ **Avoids risks of repeat cesareans** -- bowel/bladder injury, hysterectomy, and placenta accreta or previa in future pregnancies
- ✓ Especially valuable if you plan to have **more children**

RISKS TO KNOW ABOUT

- **Uterine rupture** -- the prior scar opens during labor; rare (less than 1% with one low transverse scar) but serious for mom and baby
- **Failed TOLAC** ending in cesarean -- carries higher infection and blood-loss risk than a planned repeat cesarean
- **Need for emergency cesarean**, blood transfusion, or hysterectomy
- Rare but possible: **injury to the baby** if rupture occurs
- Risks are **lower with**: prior vaginal birth, spontaneous labor, one prior low-transverse incision
- Risks are **higher with**: induction or augmentation of labor, short interval since last cesarean, more than one prior cesarean, large baby

WHY YOUR PRIOR INCISION MATTERS

- The scar on your **uterus** is what matters -- not the scar on your skin (they can differ)
- **Low transverse** (side-to-side, lower uterus) -- most common; **lowest rupture risk**; VBAC is an option
- **Low vertical** (up-and-down, lower uterus) -- **higher** rupture risk; VBAC may still be considered case-by-case
- **Classical / high vertical** (upper uterus, often used in very preterm cesareans) -- **highest** rupture risk; **VBAC is NOT recommended**
- **Please request your operative report** from any prior cesarean so we can review the exact incision type
- If records are unavailable, we will discuss the safest plan based on your history

WHO IS A GOOD VBAC CANDIDATE?

- ✓ **One** (sometimes two) prior **low transverse** cesarean(s)
- ✓ **No other uterine surgery** with full-thickness incisions (eg, certain myomectomies)
- ✓ **No prior uterine rupture**
- ✓ **Single baby**, head-down, at term
- ✓ **No medical or obstetric reason** that requires cesarean (eg, placenta previa)
- ✓ Delivering at a **hospital equipped for emergency cesarean** with anesthesia and OR available 24/7
- ✓ **Spontaneous labor** is ideal -- induction is sometimes possible but increases risk and lowers success rate

WHAT TO EXPECT IN LABOR

- ✓ **Continuous fetal heart-rate monitoring** -- the most reliable early sign of uterine rupture
- ✓ **IV access** placed on arrival in case of emergency
- ✓ **Epidural anesthesia is allowed** and does not hide rupture symptoms
- ✓ Avoid certain induction agents (eg, **prostaglandins like misoprostol**) due to higher rupture risk
- ✓ **Plans can change** -- if labor stalls, the baby is in distress, or new risks appear, we may recommend cesarean
- ✓ If you arrive in **active labor** when a repeat cesarean was planned, VBAC may still be safer at that point
- ✓ Bring your **birth preferences** -- they help guide care, even if plans shift

⚠️ CALL OR GO TO LABOR & DELIVERY IMMEDIATELY FOR

- ⚠️ Severe or sudden abdominal pain, especially between contractions
- ⚠️ A 'tearing' or 'ripping' sensation in the abdomen
- ⚠️ Vaginal bleeding heavier than spotting
- ⚠️ Sudden change in contraction pattern or loss of contractions
- ⚠️ Lightheadedness, fainting, or rapid heartbeat
- ⚠️ Leaking fluid from the vagina (possible water breaking)
- ⚠️ Any concern that something feels 'wrong' -- trust your instincts

⚠ Decreased or absent fetal movement

Your Birth, Your Choice -- With the Right Information

Both VBAC and a planned repeat cesarean are reasonable, safe options for the right candidate. There is no single 'right' answer -- the best choice depends on your medical history, this pregnancy, your future family plans, and what feels right to you. Bring your prior operative report (or let us help request it), your questions, and your birth goals to your prenatal visit. We will go through the numbers together and build a plan that puts you and your baby first.